OPERATION: Transurethral resection of bladder tumor (TURBT) with intravesical mitomycin-C instillation.

INDICATION: A 64-year-old male smoker with painless gross hematuria and computed tomography urogram demonstrating a 2.5 cm papillary lesion on the posterior-lateral bladder wall.

OPERATIVE DETAILS: Following the administration of general anesthesia and intravenous antibiotic prophylaxis, the patient was placed in the dorsal lithotomy position. A 24-French rigid continuous-flow resectoscope sheath with a 30-degree optical telescope was passed into the urinary bladder. Cystoscopy confirmed normal urethral mucosa. Systematic bladder inspection identified a solitary, 2.5 cm, frond-like, exophytic papillary tumor situated on the right posterolateral wall, approximately 2 cm superior to the right ureteral orifice. The left ureteral orifice and bladder dome were pristine.

Using a monopolar wire loop electrode with glycine 1.5% irrigating solution, the exophytic portion of the tumor was resected in thin sequential slices and collected in a separate specimen container labeled 'superficial tumor.' The resection was then extended deeply into the underlying bladder wall to harvest the muscularis propria base of the tumor, which was submitted separately labeled 'deep muscle base' to allow definitive pathological staging of muscle invasion.

Meticulous hemostasis was achieved using electrocautery coagulation across the resection crater. The right ureteral orifice was inspected and remained completely uninjured with clear peristaltic efflux. The resected fragments were thoroughly evacuated using an Ellik bulb. Final cystoscopic survey revealed a clean resection bed without evidence of bladder perforation. An 18-French two-way Foley catheter was placed into the bladder. Within two hours postoperatively, 40 mg of mitomycin C diluted in 20 mL sterile water was instilled into the bladder via the catheter, retained for one hour with periodic patient repositioning, and subsequently drained with clear effluent.